

RESPONSE TO
Request for Information:
Accelerating the Adoption and Use of Artificial Intelligence
as Part of Clinical Care

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Accelerating Trustworthy, Equitable, and Human-Centered AI in Clinical Care

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Submitted to:
Office of the Deputy Secretary
Assistant Secretary for Technology Policy (ASTP)
Office of the National Coordinator for Health IT (ONC)
U.S. Department of Health and Human Services

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Executive Summary

SANCIAN LLC submits this response to HHS's Request for Information on accelerating AI adoption in clinical care. Drawing on over 15 years of experience implementing AI governance across federal health agencies—including HHS, CDC, FDA, and NIH—we offer an operational framework for translating AI policy into sustainable clinical practice.

Core Position

HHS should treat AI not as technology to be adopted, but as care infrastructure to be governed. The gap between AI potential and AI reality is not a technology problem—it is a governance, implementation, and human factors problem.

Five Principles for Acceleration

- **Governance-first adoption.** Establish accountability structures before technology procurement. Abstract frameworks fail at the point of care; organizations need actionable playbooks.
- **Readiness before reimbursement.** Require organizations to demonstrate governance maturity as a prerequisite for AI-related payment incentives. Premature adoption without readiness wastes resources and erodes trust.
- **Payment aligned to burden reduction.** Incentivize AI that saves clinician time, reduces documentation burden, and improves access—not simply reward adoption of new technology.
- **Implementation science over model development.** Fund research on real-world deployment, workflow integration, and adoption drivers—not just algorithmic performance in controlled settings.
- **Human capacity as a success metric.** Measure cognitive load reduction, time returned to patient care, and clinician trust. AI that adds burden—even if accurate—fails the people it aims to serve.

Why Now

With accelerating federal AI strategies, workforce strain, and rising care complexity, the next 24–36 months represent a narrow window to embed governance and human-centered design before AI becomes irreversible infrastructure. Rural, safety-net, and underserved settings face the greatest risk of being left behind—or harmed—if equity is not embedded from the start.

SANCIAN's Contribution

This response offers operational frameworks HHS could pilot or scale, including: organizational readiness assessment (AI ReadyCheck™), equity impact assessment (AI-EQUITYClear™), governance maturity models, named accountability structures (AI Executive Owner), and human-centered metrics aligned to our AI SoftLife™ philosophy. We stand ready to support HHS in translating these recommendations into implementation.

About SANCIAN LLC

Who We Are

SANCIAN LLC is an evidence-based AI governance and implementation advisory focused on public health, health systems, and regulated environments. We combine validated frameworks—including NIST AI RMF, ISO/IEC 23894, PMBOK, Kotter, and ADKAR—with operational tooling for readiness assessment, equity evaluation, and sustainable adoption.

Our work spans the HHS ecosystem, including engagements with agencies such as HHS, CDC, FDA, and NIH. Our founder brings over 15 years of experience analyzing AI deployments across government and health systems, with doctoral-level expertise in public health and clinical management. This dual lens—clinical and population health—informs our approach to AI that serves both individual patients and community wellbeing.

Our Operating Model

SANCIAN Framework: Readiness → Selection → Implementation → Governance → Continuous Improvement

AI SoftLife™ Philosophy: Design AI as capacity infrastructure that reduces cognitive burden, protects clinicians' energy, and sustains humane care delivery. AI should support presence, not replace judgment.

These principles guide every recommendation in this response.

Question 1: Barriers to Private Sector Innovation and Adoption

RFI Question: What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Observed Barriers

Fragmented governance and unclear accountability. Organizations face conflicting guidance across HHS divisions, with no clarity on which agency's rules apply—particularly for non-device AI outside FDA's traditional scope. This uncertainty delays procurement and suppresses investment.

Low organizational readiness. Gaps extend beyond technology to data governance, workflow optimization, change management, and leadership alignment. Organizations underestimate the transformation required, leading to failed implementations that erode confidence in future AI initiatives.

Trust gaps across stakeholders. Clinicians, patients, and administrators express legitimate concerns about bias, safety, explainability, and liability that current deployments have not adequately addressed.

Clinician burnout and workflow friction. Consistent with our human-capacity principle, AI that adds steps or cognitive load creates friction rather than relief—regardless of technical performance. Exhausted care teams cannot absorb poorly designed tools.

Procurement complexity and reimbursement uncertainty. Traditional processes add 6–18 months to adoption timelines. Unclear ROI projections make business cases difficult to build.

SANCIAN Response

We address these barriers through AI ReadyCheck™ (organizational readiness baseline), governance-first design aligned to NIST AI RMF, workflow co-design with clinician input, and equity impact assessment (AI-EQUITYClear™) before deployment.

HHS Opportunity

Standardize readiness and governance expectations across programs. Encourage "**readiness before reimbursement**"—ensuring organizations demonstrate governance maturity as a prerequisite for AI-related payment incentives. Frameworks like those we implement at SANCIAN demonstrate how this can work operationally.

Question 2: Regulatory, Payment Policy, and Programmatic Changes

RFI Question: What regulatory, payment policy, or programmatic design changes should HHS prioritize?

Regulatory Recommendations

Create a tiered, risk-based AI governance pathway aligned with NIST AI RMF risk tiers. Lower-risk administrative AI could follow streamlined pathways; higher-risk clinical decision support would require rigorous evaluation. This provides predictability while maintaining proportionate oversight.

Issue guidance for non-device clinical AI accountability. Publish model accountability frameworks addressing roles, responsibilities, and oversight. Relevant touchpoints: ONC's Health IT Certification Program (45 CFR Part 170), Conditions of Participation (42 CFR Parts 482–485).

Enable pilots with real-world evidence requirements. Structured pilots that integrate AI into care pathways with mandatory evidence collection would build the evidence base for broader adoption.

Reimbursement Recommendations

Consistent with our payment-alignment principle, CMS should incentivize AI that delivers measurable value:

- **Clinician time saved** — Reward AI that reduces documentation and administrative burden.
- **Improved access and continuity** — Support AI that expands access to underserved and rural populations.
- **Value-based AI add-ons** — Enable payment adjustments under MSSP, ACO REACH, and bundled payments when AI contributes to outcomes.

The current fee schedule (42 CFR Part 414) requires new CPT codes or modifiers for AI-assisted services. Our governance playbooks are designed for alignment with emerging requirements.

Question 3: Novel Legal and Implementation Issues

RFI Question: For non-medical devices, what novel legal and implementation issues exist and what role should HHS play?

Key Issues

Shared liability. Liability allocation among AI developers, healthcare organizations, and clinicians remains unclear. Traditional malpractice frameworks assume human decision-makers—AI disrupts these assumptions.

Auditability of adaptive models. Organizations need mechanisms to audit model behavior, track changes, and demonstrate deployed models match validated versions.

Data provenance, consent, and security. Questions persist regarding training data sources, patient consent, and AI-specific cybersecurity vulnerabilities. HIPAA (45 CFR Parts 160, 164) requires interpretation for AI contexts.

Indemnification gaps. Standard vendor contracts do not adequately address AI-specific risks including performance guarantees, bias warranties, and monitoring obligations.

SANCIAN Approach

We implement role-based accountability matrices, continuous monitoring with model risk logs, and governance charters embedded in operations—not as standalone policy documents.

HHS Role

Convene cross-sector standards development for liability and auditability. Publish reference governance architectures. Provide model contract provisions or safe harbor guidance. HHS could pilot reference architectures such as those we implement to demonstrate operational feasibility.

Question 4: AI Evaluation Methods

RFI Question: What are the most promising evaluation methods, metrics, and human-centered evaluation methods? Should HHS support these?

What Works

Pre-deployment: Bias testing across demographic subgroups, scenario-based validation, usability testing, and "shadow mode" deployment comparing AI recommendations to actual clinical decisions.

Post-deployment: Outcome drift monitoring, safety signal surveillance, human override tracking, and periodic re-validation.

Workflow impact: Traditional accuracy metrics are necessary but insufficient. Clinical AI must be evaluated on operational integration—not just algorithmic performance.

AI SoftLife™ Metrics

Consistent with our human-capacity principle, we recommend measuring:

- **Cognitive load reduction** — Does the AI decrease mental effort?
- **Time returned to patient care** — Does the AI free clinicians for presence?
- **Trust and adoption** — Do clinicians actually use and trust the AI?

We also recommend structured equity evaluation using frameworks such as AI-EQUITYClear™ to assess algorithmic fairness across demographic subgroups.

HHS Support

Fund applied evaluation frameworks. Support shared testbeds and benchmark datasets—particularly for rural and safety-net settings where evidence is thinnest. Public-private partnerships could validate methodologies across contexts.

Question 5: Supporting Private Sector Activities

RFI Question: How can HHS best support accreditation, certification, and credentialing?

The Need

The market needs third-party validation of governance maturity, bias and safety controls, and operational integration. Healthcare organizations lack reliable signals to distinguish quality AI products from marketing.

Proposal: AI in Clinical Care Maturity Model

HHS should support development of standardized assessment frameworks for:

- **Organizations** — Readiness to adopt, implement, and govern AI responsibly.
- **Vendors** — Product quality, transparency, safety, and governance features.
- **Care pathways** — Effectiveness of AI integration into specific clinical workflows.

Additional Mechanisms

Recognize private standards from NIST, IEEE, and healthcare bodies by reference.

Provide regulatory safe harbors for certified AI. **Fund workforce credentialing** through HRSA for AI competencies.

Our governance maturity and readiness scoring provide a foundation for organizational assessment components of such models.

Question 6: AI Performance and Opportunities

RFI Question: Where has AI met or fallen short of expectations? What novel tools have greatest potential?

Where AI Works

Documentation automation (ambient intelligence, AI-assisted notes), **imaging triage** (radiology, pathology screening), **risk stratification** (sepsis prediction, early warning), and **population health analytics** (care gap identification, resource allocation).

Where AI Falls Short

Workflow misalignment creates friction. **Alert fatigue** leads to override of valid recommendations. **Black-box decisioning** undermines trust. **Equity gaps** from non-representative training data worsen disparities in underserved populations.

High-Potential Applications

Care navigation (guiding patients through complex journeys), **social determinants integration** (connecting patients to community resources), **chronic disease coaching** (personalized self-management), **team-based decision support**, and **frailty/dementia care** for aging populations.

Consistent with our AI SoftLife™ philosophy: ***AI should support presence, not replace judgment.*** The highest-value applications restore clinician capacity to be fully present with patients.

Question 7: Decision-Makers and Administrative Hurdles

RFI Question: Which roles have most influence on AI adoption? What are the primary administrative hurdles?

The Reality

Multiple stakeholders hold effective veto power: CIO/CTO (infrastructure), CMIO/CMO (clinical safety), Compliance/Legal (regulatory exposure), and CFO (budget). The result: **no single owner of AI outcomes**. Decisions stall. Accountability diffuses. Promising initiatives die in committee.

Primary Hurdles

Procurement and IT security cycles add 6–18 months. Absence of governance structures forces ad hoc decisions. Medical staff bylaws rarely address AI. Legal review delays from lack of AI expertise. Change management underinvestment undermines adoption.

SANCIAN Solution

- **Named AI Executive Owner model** — Single accountable leader with cross-functional authority.
- **Cross-functional AI governance councils** — Formalized structures with defined decision rights.
- **Decision clarity before tooling** — Governance infrastructure before product evaluation.

This reflects our governance-first principle. HHS should encourage accountable leadership models in guidance and grants—including Conditions of Participation updates requiring AI oversight designation.

Question 8: Interoperability for AI Development

RFI Question: Where would enhanced interoperability widen opportunities and accelerate AI development?

Priority Data Types

FHIR-based clinical data with bulk access for population AI. **Social determinants of health** (currently inconsistent). **Patient-reported outcomes** captured outside encounters. **Imaging and genomic data** for precision medicine applications.

Standards and Benchmarking

Model cards and audit metadata for standardized AI documentation. **Benchmark datasets for fairness** representing diverse populations. **Provenance tracking** for data lineage and model versioning.

SANCIAN View

Interoperability must include governance data, not just clinical data. Standards for AI oversight artifacts—audit logs, performance results, bias assessments—would enable consistent governance and regulatory oversight.

HHS should fund interoperability for AI oversight artifacts and require transparency standards in funded programs. USCDI expansion should prioritize AI-relevant data elements.

Question 9: Patient and Caregiver Perspectives

RFI Question: What challenges do patients and caregivers want AI to address? What concerns do they have?

What Patients Want

Access (timely appointments, expanded availability). **Continuity** (coordinated care across providers). **Clarity** (understandable information, timely results). **Less friction** (reduced paperwork, streamlined processes). **Personalized but fair care** (tailored treatment without discrimination).

Patient Concerns

Bias and exclusion (fear of discrimination). **Loss of human connection** (AI replacing empathy). **Data misuse** (uncertainty about privacy). **Lack of recourse** (unclear accountability for harm). **Accessibility** (barriers for those with disabilities or limited digital access).

AI SoftLife™ Response

Human-in-the-loop by design. Transparency as trust infrastructure. Equity checks embedded early. HHS should make patient trust a core success metric—not a soft outcome, but the foundation of sustainable adoption.

Question 10: AI Research Priorities

RFI Question: What AI research areas should HHS prioritize? What does the literature say?

High-Impact Research Areas

Consistent with our implementation-science principle:

- **Implementation science in real clinics** — How to deploy effectively, including change management and adoption drivers.
- **Equity and bias mitigation in deployment** — Detecting and addressing bias as AI operates in practice.
- **Human-AI teaming** — Optimal collaboration patterns and trust calibration.
- **Safety and drift monitoring** — Detecting failures and ensuring robustness across contexts.
- **Economic impact of burden reduction** — Effects on clinician time, costs, and workforce sustainability.

Literature Gap

Plenty on models. Not enough on adoption, outcomes, and workflow impact. Most studies evaluate controlled settings; evidence on real-world implementation is thin. Rural and safety-net settings are particularly underrepresented.

HHS Opportunity

Fund longitudinal pilots tied to outcomes and workforce wellbeing. Support public-private learning collaboratives for sharing implementation lessons—including failures. This underscores the implementation gap noted throughout this response.

Closing Position

Our Recommendation

HHS should treat AI not just as technology, but as care infrastructure—governed, evaluated, reimbursed, and funded to:

- **Improve outcomes** for patients across all populations, including rural and safety-net settings.
- **Restore clinician capacity** by reducing burden rather than adding it.
- **Advance equity** by embedding fairness from design through deployment.
- **Build durable public trust** through transparency, accountability, and human-centered design.

Why SANCIAN's Lens Matters

Because adoption fails when governance is abstract and care teams are exhausted.

AI succeeds when it is designed for clarity, accountability, and human ease.

Final Message

Accelerate AI by making it operationally safe, economically viable, and emotionally sustainable for the people who deliver and receive care.

The next 24–36 months represent a narrow window. SANCIAN LLC stands ready to support HHS in translating these recommendations into operational reality.

Respectfully submitted,

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